Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bloods / symptom review	Blood count, kidney and liver profile; review medicines and bleeding risk	Confirm Day 15 loading visit; assess injection tolerability	Liver profile when indicated; review bleeding and nerve symptoms	Clinical and imaging review of benefit and tolerability	Follow unresolved symptoms or test abnormalities
electrocardiogram (ECG) / echo	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA)	Assess personal fracture / osteoporosis risk	DXA only when indicated	No cycle-based DXA appointments	Repeat only according to the bone-risk plan	Continue bone care according to risk
Advanced-disease response	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Oestrogen blood tests	Tell the team that fulvestrant is planned	Some tests can read falsely high	Confirm a suitable test with the laboratory if needed	Do not change treatment on a misleading result	Tell later clinicians about prior fulvestrant

THE WHOLE TREATMENT JOURNEY

Fulvestrant · injections every 4 weeks: treatment and checks

Postmenopausal ER-positive locally advanced or metastatic breast cancer

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00361	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00361	Blood count, kidney and liver profile; review medicines and bleeding risk	Confirm Day 15 loading visit; assess injection tolerability	Liver profile when indicated; review bleeding and nerve symptoms	Clinical and imaging review of benefit and tolerability	Follow unresolved symptoms or test abnormalities
ECG / echo NCCP 00361	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / DXA NCCP 00361	Assess personal fracture / osteoporosis risk	DXA only when indicated	No cycle-based DXA appointments	Repeat only according to the bone-risk plan	Continue bone care according to risk
Advanced-disease response NCCP 00361	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Oestrogen blood tests NCCP 00361	Tell the team that fulvestrant is planned	Some tests can read falsely high	Confirm a suitable test with the laboratory if needed	Do not change treatment on a misleading result	Tell later clinicians about prior fulvestrant

What each check is for

1 **Confirm the plan**

Before starting or changing treatment

Sources: NCCP 00361

2 **Checks and symptoms**

At the agreed review points

Sources: NCCP 00361

3 **Review ongoing benefit**

At setting-specific decision points

Sources: NCCP 00361

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25

Source reconciled; independent clinical, pharmacy and patient/PPI review pending